

FIRST AID



FIRST AID RANGES from cleaning a small wound and covering it with sticking plaster, to dealing with serious injuries at a major disaster. But its main aims are the same: to save life, prevent the casualty's condition from worsening, promote healing and recovery, and arrange for expert help at the earliest opportunity. Recently, first aid has advanced greatly due to a better understanding of the body's needs in serious injury or disease, improved medical equipment, and mobile communications. It now plays an even more vital role in saving lives, and speeding a casualty's recovery.

ABC of first aid

ABC stands for the body's three vital needs. "A" stands for airway: the airway needs to be open so that oxygen-containing air can enter the lungs. "B" stands for breathing, by which the body inhales fresh air and expels stale air. "C" stands for the circulation of the blood, which distributes oxygen around the body. When dealing with an unconscious casualty, the first aider must check that the casualty has a clear airway, is breathing, and has a pulse that indicates blood circulation.



Airway

Inhaled foreign bodies or fluid can block the airway. By tipping the head back and straightening the airway, a first aider can look for blockages.



Breathing

If breathing stops, the first aider may blow air at regular intervals through the casualty's mouth into their lungs. This is called artificial ventilation.



Circulation

If a pulse is absent, the first aider may carry out heart massage (external chest compression) to try to stimulate the heart into action.

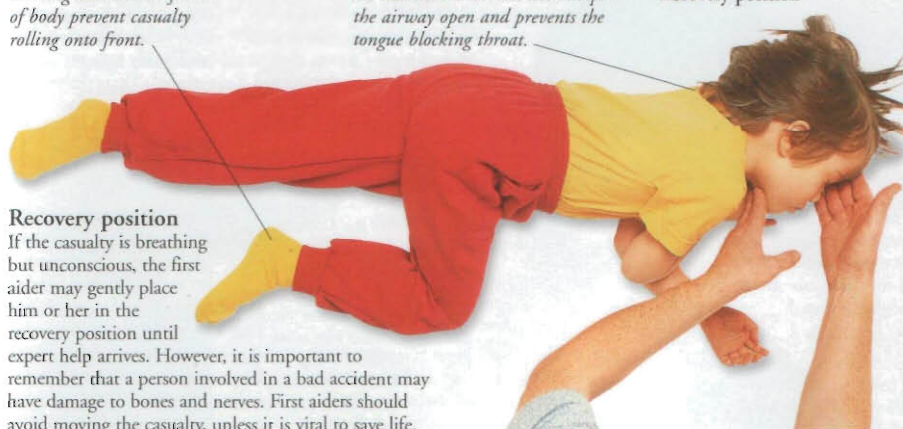
Bent leg and arm in front of body prevent casualty rolling onto front.

An extended head and neck keeps the airway open and prevents the tongue blocking throat.

Recovery position

Recovery position

If the casualty is breathing but unconscious, the first aider may gently place him or her in the recovery position until expert help arrives. However, it is important to remember that a person involved in a bad accident may have damage to bones and nerves. First aiders should avoid moving the casualty, unless it is vital to save life.



Scene of a motorcycle accident

At the scene

Effective first aid – the temporary treatment of injury or illness while waiting for medical aid – relies on correctly assigning priorities. At an accident, one of the first priorities is to summon the emergency services.

Assessing conditions

Experienced first aiders know that noisy casualties are not necessarily the most hurt. At a multiple accident, they assess quickly the condition of all casualties, then concentrate available first aid on the most seriously injured. In hospitals, this assessment is known as triage.

Raising the alarm

A telephone call is usually the best way to get help. Special telephones are located in areas such as motorways, but calls to the emergency services are free on all telephones. Shouting, waving flags, or firing flares are all alternative methods.



Spanish public telephone

Further danger

First aiders should never place themselves or others in danger. Before treating the casualty, they should try to make the area safe. Fire, traffic, electricity, and unsafe structures are some of the hazards that may delay treatment.



Fire extinguisher

First-aid kit

In many countries, the law requires workplaces and schools to keep first-aid kits. The contents should be kept in a clean, marked container and be re-stocked regularly. Because there is a danger of misusing drugs, the contents of some kits are restricted to sterile wound dressings.



Safety scissors

Forceps (tweezers)

Painkillers

Cottonwool or similar absorbents

Elasticated bandage for treating sprains

Bandages and sterile dressings

Eye bath

First-aid kit for use in the home

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